

Jeff Bridges

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Health assessment report

Harmful information

No harmful information has been included in this report

Assessment details	Date of birth	20 February 1959
	Benefit	Personal Independence Payment
	Application submitted	19 March 2024
	Date of assessment	5 April 2024
	Date report completed	8 April 2024
	Assessment type	Assessment report without scores or review dates Face-to-face assessment Paper based assessment Telephone assessment Full assessment (Unknown type needed)
	Report type	Full assessment (Unknown type needed)
	LOT	#0001

Health professional	Gary Busey
Evidence used	Evidence type A
	Evidence type B
	Evidence type C
Other evidence used	Open text field
Evidence requested but not returned	Open text field
Other evidence requested but not returned	Open text field
Consultation	Consultation date Date Month Year
	Identity confirmed Yes / No
	How identity was confirmed Method
	Consent given Yes / No
	Additional attendee name

Name Name

Relationship or role

Name Name

Additional attendee name

Name Name

Relationship or role

Name Name

Condition history

Condition name

When did the condition or disability start?

Answer

Diagnosis, impact and variability

Answer

Current treatments, specialists input and hospital stays

Answer

Home Therapies [Conditional]

Answer

Suicidal thoughts and self harm [Conditional]

Answer

(Legacy) The scope of the assessment was explained to the claimant and the claimant gave verbal consent to proceed.

Fibromyalgia (Legacy)

He was diagnosed by his GP in 2012 and started on pain relief. He tried different medications for pain which did not work. He then took Tramadol (pain relief) for 4 years, he then moved to America for a while, took pain relief there but came off this. He no longer takes Tramadol but does use over the counter pain relief.

He was referred to pain management but did not attend as the referral was lost - he states that he has since been re-referred to the pain management service 1 month ago.

He states that his GP has discussed him being referred to weight managed as he nearly obese on the weight scale (BMI - body mass index). He has not been referred to any other services.

Day to day he experiences symptoms of forgetfulness - fibro fog, feeling overpowered with pain in his shoulders (worse on the right side), lower right side of his back and hip as well as both ankles where he has a lot of tension when he walks. His knees and elbows are ok.

He suffers with fatigue also but is not sure what is related to his sleep apnoea. He feels that his symptoms have worsened over time, more so since 2021. He reports no variability. He has not needed to stay in hospital as a result of this.

Myofacial pain syndrome (long term pain condition were pressure on areas - trigger points, causes pain). He was diagnosed by a specialist last year. He was referred to an orthodontist due to him wearing a mouth guard for 20 years but there is no other treatment planned. He experiences pain symptoms - he is unable to differentiate this from his fibromyalgia pain and this follows the same above history. He takes over the counter pain relief but feels that his left sided jaw pain is the worst.

Other conditions

No other medical problems reported during the assessment.

Current medication and treatment

Medication name

Dosage

Answer

Frequency

Answer

Reason for taking

Answer

Effectiveness [Conditional]

Answer

Side Effects [Conditional]

Answer

(Legacy) He is doing home physio exercises himself at night time before bed time. Uses CPAP machine 2-3 nights a week for sleep apnoea. Has not used this for a month. He is able to set this up and use it himself. He does not use this as he knows it is not working as he needs the BIPAP.

Social and occupational history

Accommodation: He lives in a flat - second floor, accessed by stairs. He lives by himself. The shower is over the bath.

Aids and adaptation: None

Education: He went to a normal school, had coloured filters for reading and extra time in exams. He then attended university after A 'levels and a foundation degree. He used dragon software to read things out for him in lessons whilst at university for his bachelor degree. He studied software engineering as a bachelor degree. He then did an Masters in Business Administration (MBA) and graduated in 2021 - he was supported by his Masters Tutors as per masters degree candidates and this was all coursework online. He did not use any aids whilst doing his MBA.

Work: He stopped working in December 2023, he was a programme manager for Amazon full time and prior to this a data analyst and worked in analytics over the years, including for Google. He left his role in December as he was performing poorly. Amazon did not provide him with any occupational health input and let him go without supporting him due to his dyslexia causing fatigue. He is currently looking for work, has applied for over 100 jobs, with blue chip companies and has had 3 interviews so far. He states the job market is currently tough.

Driving: He has a driving licence but was advised not to drive by the sleep apnoea service due to his ongoing tests in 2022. He stopped driving in 2012 as he did not need a car as he lived in central London.

How else do they get around: He uses public transport and walks places. As he is not working he stays at home more but is he has to go out anywhere he will use the underground and buses or walks places.

Pets: None

Caring responsibilities: None

Hobbies/pastimes: He plays chess on his phone to keep his brain active. He tries to read in his spare time. He is looking for work and applying for jobs constantly. He likes to spend his time exercising, on the cross trainer and he has accessed courses as he is on UC, an engineering one to give himself more employment opportunities. He likes to go for walks.

Housework: He does do housework and tries to not let the place get too messy. He does his own laundry and dishes. Hoovering is a problem due to back pain. He does do this and does clean his bath but has back pain.

Shopping: It is too difficult to carry shopping due to pain in his back and has food deliveries. When this comes he puts this away himself.

Who completed the questionnaire: Himself.

Any formal or informal support received: None and manages himself.

How do they spend their day, any difference at the weekends? He spends time with his partner at the weekends at times and during the weekdays.

Physical examination

Consent to physical examination
Yes / No

Why did they not give consent
Open text field

Chaperone name
Name Name

Physical examination notes

Mental state observations

IOs showed no that he did not appear fatigued or tired during the assessment. There was no expression of pain, breathlessness or wheeze.

There is no other evidence to support restriction.

This is consistent with the lack of reported restriction in the questionnaire and the ability reported in the FH.

It is likely that he can stand and then move more than 200 metres, either aided or unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

Informal observations

Did not sound anxious during the assessment. Did not require prompting.

Manner appeared normal and did not appear withdrawn. Mood appeared stable.

Had adequate rapport

Had adequate insight.

Had adequate memory.

Did cope with the assessment despite the prolonged length of it and being assessed alone.

Speech was normal in tone and content. Volume was normal and comprehensible.

No evidence of a cognitive impairment.

Engaged with the assessment process without any input.

(Telephone assessment undertaken)

The telephone assessment lasted 2 hours and 20 minutes. He did not appear fatigued or tired during the assessment.

There was no expression of pain, breathlessness or wheeze.

Daily living findings

Preparing food

He will prepare things like pasta, cuts up some onions normally. He makes himself quite simple meals and relies on himself for this only, preferring it this way.

He has had one incident where he has cut his index finger where he needed a plaster whilst chopping. States this was due to a muscle flinch. He does not use any aids. he does not need prompting. He is able to make teas and coffees, using the kettle.

How well can they prepare food?

(1a) can prepare a cook a simple meal unaided

Justification for preparing food

PIP2 - Reports issues with muscle flinching in his right arm.

HOC shows that he is under GP care for his back and fibromyalgia, having just been referred to pain clinic. MT shows that he takes no prescribed pain relief and only OTC anti inflammatory pain and as required mild pain relief.

HOC shows arm flinching but there is no FE to support significant restriction with this. FH in activity 4 - washing and bathing, shows that he still wet shaves himself which is unlikely to support significant risk from any flinching when there is no reported trigger. The above FH reports that he has just cut himself once as a result of his flinching when preparing food in the kitchen.

HOC shows that he has sleep apnoea and whilst he reports fatigue as an issue, he is only needing to take a 20 minute nap in the day normally to feel refreshed. SOH shows that he likes to exercise and go for walks. It also shows that is doing housework, is looking for work and was working full time until 4 months ago having lost his job due to his performance and he did not have any occupation health input.

IOs showed no that he did not appear fatigued or tired during the assessment. There was no expression of pain, breathlessness or wheeze.

HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of Autism and although he had support at school, this was for his dyslexia and he has been able to obtain a MBA in recent years without any significant support or aids.

SOH shows that he has been able to achieve a degree and a post grad MBA. He has been working in responsible roles within blue chip companies.

Although he lost his job in December 2023, he did not have any occupational health input prior to this. There was no evidence in his MSE to show any restriction from a mental health or cognitive perspective. There is no other evidence to support restriction.

Any reported restriction in his questionnaire is likely to be medically

inconsistent. It is likely that he can prepare and cook a simple meal unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

Taking nutrition

How well can they take nutrition

(2a) Can take nutrition unaided

Justification for taking nutrition

He is able to take drinks himself. He does eat and drink by himself, does not use any adapted cutlery, does not need prompting and has no issues swallowing or chewing other than biting his lip or cheek. He does not need to be prompted.

PIP2 - he bites into his tongue and cheeks due to temporomandibular joint pain leading to jaw soreness.

There is no HOC to support dietetic support at present and there is no reported weight loss. He takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of autism. There was no evidence in his MSE to show any restriction from a mental health or cognitive perspective. There is no other evidence to support restriction.

Whilst HOC shows Temporomandibular joint pain and referral to an orthodontist, he has not been to A&E since 2003 with any oral injuries despite reporting that he bites his lip daily when eating. As above, there is no evidence of dietetic support for this, special diets or alternative food administration routes.

Managing therapy or monitoring a health condition

How well can they manage therapy or monitor a health condition?

(3a) Either does not receive medication or therapy or need to monitor a health condition, or can manage medication or therapy or monitor a health condition unaided

Justification for managing therapy or monitoring a health condition

HOC shows arm flinching but there is no FE to support significant restriction with this. FH in activity 4 - washing and bathing, shows that he still wet shaves himself which is unlikely to support significant risk from any flinching when there is no reported trigger.

This is medically consistent with the FH which shows no significant restriction.

It is likely that he can take nutrition unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

He takes his own medication himself and requires no prompting, assistance and uses no aids.

PIP2 - reports no issues.

HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of autism. There was no evidence in his MSE to show any restriction from a mental health or cognitive perspective. There is no other evidence to support restriction.

MT shows that he takes no prescribed medication and OTC pain relief only. He is independent with his home physio exercises as he is with his CPAP machine. Whilst HOC shows Temporomandibular joint pain and referral to an orthodontist, he has not been to A&E since 2003 with any oral injuries despite reporting that he bites his lip daily when eating.

HOC shows arm flinching but there is no FE to support significant restriction with this. FH in activity 4 - washing and bathing, shows that he still wet shaves himself which is unlikely to support significant risk from any flinching when there is no reported trigger. This is medically consistent with the FH and questionnaire which shows no significant restriction.

It is likely that he either does not receive medication or therapy or need to monitor a health condition, or can manage medication or therapy or monitor a health condition unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard

Washing and bathing

How well can they wash and bathe?

(4a) Can wash and bathe unaided

Justification for washing and bathing

He is able to get into the bath himself, then washes himself and gets out of the time bath himself. He does wet shave himself normally and does not normally cut himself as a result of flinching. He requires no prompting, has no assistance and uses no aids.

PIP2 - reports problems with balance. Suffers with stiffness and pain.

HOC shows that he is under GP care for his back and fibromyalgia, having just been referred to pain clinic. MT shows that he takes no prescribed pain relief and only OTC anti inflammatory pain and as required mild pain relief.

HOC shows arm flinching but there is no FE to support significant restriction with this. FH shows that he still wet shaves himself which is unlikely to support significant risk from any flinching when there is no reported trigger.

HOC shows that he has sleep apnoea and whilst he reports fatigue as an issue, he is only needing to take a 20 minute nap in the day normally to feel refreshed. SOH shows that he likes to exercise and go for walks. It also shows that is doing housework, is looking for work and was working full time until 4 months ago having lost his job due to his performance and he did not have any occupational health input.

IOs showed no that he did not appear fatigued or tired during the assessment. There was no expression of pain, breathlessness or wheeze.

HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of Autism and although he had support at school, this was for his dyslexia.

There was no evidence in his MSE to show any restriction from a mental health or cognitive perspective.

This is consistent with the lack of reported restriction in his FH.

The reported restriction in the questionnaire is likely to be medically inconsistent.

There is no other evidence to support restriction.

It is likely that he can wash and bathe unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

Managing toilet needs or incontinence

Reports no issues with getting on and off the toilet, cleaning himself and does not have any incontinence or wear incontinence pads. He does not require prompting.

How well can they manage toilet needs or incontinence?

(5a) Can manage toilet needs or incontinence unaided

Justification for managing toilet needs and incontinence

PIP2 - reports no issues.

HOC shows that he is under GP care for his back and fibromyalgia, having just been referred to pain clinic. MT shows that he takes no prescribed pain relief and only OTC anti inflammatory pain and as required mild pain relief.

HOC shows arm flinching but there is no FE to support significant restriction with this. FH shows that he still wet shaves himself which is unlikely to support significant risk from any flinching when there is no reported trigger.

HOC shows that he has sleep apnoea and whilst he reports fatigue as an issue, he is only needing to take a 20 minute nap in the day normally to feel refreshed. SOH shows that he likes to exercise and go for walks. It also shows that is doing housework, is looking for work and was working full time until 4 months ago having lost his job due to his performance and he did not have any occupational health input.

IOs showed no that he did not appear fatigued or tired during the assessment. There was no expression of pain, breathlessness or wheeze.

HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of Autism and although he had support at school, this was for his dyslexia. There was no evidence in his MSE to show any restriction from a mental health or cognitive perspective.

This is consistent with the lack of reported restriction in his FH and questionnaire.

There is no other evidence to support restriction.

It is likely that he can manage toilet needs or incontinence unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

Dressing and undressing

He will find getting his socks on more difficult as his hamstrings are tight due to his fibromyalgia and he finds himself off balance needing to sit on the bed to dress. He will still dress himself despite pain, leaning forward as required. He requires no aids, assistance of prompting to dress or with choice of clothing.

How well can they dress and undress?

(6a) Can dress and undress unaided

Justification for dressing and undressing

PIP2 - reports no issues.

HOC shows that he is under GP care for his back and fibromyalgia, having just been referred to pain clinic. MT shows that he takes no prescribed pain relief and only OTC anti inflammatory pain and as required mild pain relief.

HOC shows arm flinching but there is no FE to support significant restriction with this. FH shows that he still wet shaves himself which is unlikely to support significant risk from any flinching when there is no reported trigger.

HOC shows that he has sleep apnoea and whilst he reports fatigue as an issue, he is only needing to take a 20 minute nap in the day normally to feel refreshed. SOH shows that he likes to exercise and go for walks. It also shows that is doing housework, is looking for work and was working full time until 4 months ago having lost his job due to his performance and he did not have any occupation health input.

IOs showed no that he did not appear fatigued or tired during the assessment. There was no expression of pain, breathlessness or wheeze.

HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of Autism and although he had support at school, this was for his dyslexia.

There was no evidence in his MSE to show any restriction from a mental health or cognitive perspective.

This is consistent with the lack of reported restriction in his questionnaire.

The reported restriction in the FH is likely to be medically inconsistent and is likely to be mitigated by sitting down to dress which is not in scope.

There is no other evidence to support restriction.

It is likely that he can dress and undress unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

Communicating verbally

He states he gets by daily with communicating with people. He does not use any aids. He is better when he is at work but states that people used to ask him to repeat himself. He has been told that he mumbles and slurs his words. He talks too fast. He gets by, repeating himself. He does struggle engaging in loud environments as he cannot speak loud enough.

How well can they communicate verbally?

(7a) Can express and understand verbal information unaided

Justification for communicating verbally

PIP2 - reports issues due to obstructive sleep apnoea, resulting in slurred speech and due to a mouth guard that he has been wearing for 20 years, this has affected his speech. HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of Autism and although he had support at school, this was for his dyslexia and he has been able to obtain a MBA in recent years without any significant support or aids.

SOH shows that he has been able to achieve a degree and a post grad MBA. He has been working in responsible roles within blue chip companies. Although he lost his job in December 2023, he did not have any occupational health input prior to this. There was no evidence in his MSE to show any restriction from a mental health, communication or cognitive perspective.

Whilst HOC shows a history of Temporomandibular (jaw) joint pain and fatigue, IO showed that he was communicative, was talkative, spoke in context, was easily understood, spoke at a normal rate and was able to understand the assessors questions. This was despite the assessment taking 2 hours and 20 minutes.

There is no other evidence to support restriction.

The reported restriction in the questionnaire and FH is likely to be medically inconsistent. It is likely that he can express and understand verbal

information unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

Reading and understanding signs,symbols and words

He states that he used to use a screen filter and dragon software and does not use any aids at all now. He is able to read unaided for 20 minutes without the need to stop. He uses no aids or specialist software when he is using his lap top when applying for jobs.

How well can they read and understand signs, symbols and words?

(8a) Can read and understand basic and complex written information either unaided or using spectacles or contact lenses

Justification for reading and understanding signs, symbols and words

PIP2 - reports no issues.

HOC shows no formal diagnosis of Autism and although he had support at school, this was for his dyslexia and he has been able to obtain a MBA in recent years without any significant support or aids.

SOH shows that he has been working in responsible roles within blue chip companies. Although he lost his job in December 2023, he did not have any occupational health input prior to this.

There was no evidence in his MSE to show any restriction from a mental health or cognitive perspective.

Whilst SOH shows the use of aids whilst at school and the use of software for his dyslexia, FH shows no recent use of these.

There is no other evidence to support restriction.

This is consistent with no restriction being reported in the questionnaire and the ability to complete this task unaided in his FH.

It is likely that he can read and understand basic and complex written information either unaided or using spectacles or contact lenses, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

Engaging with other people face to face

He spends time with his girlfriend. He will attend appointments by himself and is able to engage with people on the way whilst using public transport and will have a conversation with them if they ask for directions. He was able to engage with people at work. He gets on better engaging with people in general if he is outdoors as they can hear him better. He has been engaging with people face to face for interviews without support as well as the job centre and meeting with the advisor unaided.

How well can they engage with other people face to face?

(9a) Can engage with other people unaided

Justification for engaging face to face

PIP2 - reports that he has a depressive disorder and has been been through an assessment for autism.

Making budgeting decisions

How well can they make budgeting decisions?

(10a) Can manage complex budgeting decisions unaided

Justification for making budgeting decisions

HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of Autism and although he had support at school, this was for his dyslexia.

SOH shows that he has been working in responsible roles within blue chip companies. Although he lost his job in December 2023, he did not have any occupational health input prior to this.

There was no evidence in his MSE to show any restriction from a mental health, communication or cognitive perspective.

IO showed that he was communicative, was talkative, spoke in context, was easily understood, spoke at a normal rate and was able to understand the assessors questions.

This was despite the assessment taking 2 hours and 20 minutes.

There is no other evidence to support restriction.

This is overall consistent with the lack of restriction indicated on the FH.

Any reported restriction in the questionnaire is likely to be medically inconsistent.

It is likely that he can engage with other people unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

He looks after his own finances and money without any support or help and is able to budget accordingly.

PIP2 - reports no issues.

HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of Autism and although he had support at school, this was for his dyslexia.

SOH shows that he has been working in responsible roles within blue chip companies. Although he lost his job in December 2023, he did not have any occupational health input prior to this. He has been able to complete a MBA in recent years.

There was no evidence in his MSE to show any restriction from a mental health or cognitive perspective.

This is overall consistent with the lack of restriction indicated on the FH and questionnaire.

It is likely that he can manage complex budgeting decisions unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly.

Mobility findings	Planning and following journeys
	How well can they plan and follow journeys? (11a) Can plan and follow the route of a journey unaided
	Justification for planning and following journeys

He is able to plan his own journeys when he has to. He can use buses, trains and undergrounds himself and does not need to be supported. He can speak and engage with people on the way and can manage diversions in the underground.

PIP2 - reports no issues.

HOC shows that he takes no medication for this mental health, is under GP care and there are no further plans for treatment. There is no formal diagnosis of Autism and although he had support at school, this was for his dyslexia.

SOH shows that he has been working in responsible roles within blue chip companies. Although he lost his job in December 2023, he did not have any occupational health input prior to this. He uses public transport and walks places. As he is not working he stays at home more but if he has to go out anywhere

Moving around

He will use the underground and buses or walks places.

How well can they move around?

(12a) Can stand and then move more than 200 metres, either aided or unaided

Justification for moving around

There was no evidence in his MSE to show any restriction from a mental health, communication or cognitive perspective.

There is no other evidence to support restriction.

This is overall consistent with the lack of restriction indicated on the FH and in the questionnaire.

It is likely that he can plan and follow the route of a journey unaided, on the majority of days, ensuring safety, timeliness, that it is done to an acceptable standard and repeatedly

He will go for walks 2 days a week lasting 20-60 minutes at a time. He does this without needing to stop. On the other days will use the cross trainer for 30-45 minutes at the gym 3 times a week (these are variable intensity workouts - ramping up to high intensities). This is a 10 minute walk each

way week to the gym and then returning from the gym. He can do this without needing to rest. He uses no aids and states that he tends to walk at an above average pace.

PIP2 - reports no issues.

PIP2 - Reports issues with muscle flinching in his right arm.

HOC shows that he is under GP care for his back and fibromyalgia, having just been referred to pain clinic. MT shows that he takes no prescribed pain relief and only OTC anti inflammatory pain and as required mild pain relief.

HOC shows that he has sleep apnoea and whilst he reports fatigue as an issue, he is only needing to take a 20 minute nap in the day normally to feel refreshed. SOH shows that he likes to exercise and go for walks. It also shows that is doing housework, is looking for work and was working full time until 4 months ago having lost his job due to his performance and he did not have any occupational health input.

Daily living qualifying period and prospective test

Have they had the functional restriction for at least 3 months?

No daily living restriction found

Daily living qualifying period detials

Open text field

Are they likely to have the functional restriction in 9 months?

No daily living restriction found

Daily living prospective test detials

Open text field

Mobility qualifying period and prospective test

Have they had the functional restriction for at least 3 months?

No mobility restriction found

Mobility qualifying period detials

Open text field

Are they likely to have the functional restriction in 9 months?

No mobility restriction found

Mobility prospective test detials

Open text field

Recommendation

Recommended future review period

X

Reason for this review period

XXX

Is it likely the functional restriction will still be present when reviewed?

XXX

Additional support

Do they need additional support for future claims processes?

Yes / No

Special rules

Do they meet the criteria for special rules?

Yes / No

XXX



[Clear data \(/manage-prototype/clear-data\)](/manage-prototype/clear-data)

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